

ADRABETADEX Document C

HCPCS coding and OPPS transitional pass-through pathway for launch

Prepared for client discussion. Content is grounded in the supplied source pack and current CMS, HRSA, Medicaid, Medicare, and public payer materials reviewed on March 30, 2026.

Why coding and pass-through determine launch cash conversion

ADRABETADEX launches as a clinician-administered product with a high per-administration cost and a route of administration that sits squarely inside hospital outpatient and specialist-center billing workflows. In that setting, coding is not an administrative tail event. Coding determines whether the claim is intelligible to the payer, whether the drug is separately payable, and how quickly providers get comfortable carrying inventory.

The source pack is correct to treat a permanent product-specific HCPCS code and transitional pass-through as separate but related workstreams. CMS states that OPPS drug and biologic pass-through approval is **not contingent on prior assignment of a national HCPCS code**, which means the manufacturer can pursue pass-through without waiting for the permanent J-code to go live.

HCPCS Level II path

- CMS now processes HCPCS Level II drug and biologic coding decisions on a quarterly cycle.
- CMS' first-quarter 2025 coding file shows that a first-quarter drug application produced new codes effective July 1, 2025. The same quarterly structure makes the first complete post-approval ADRABETADEX application cycle the key date to hit.
- With a supplied PDUFA date of Monday, August 17, 2026, the first full quarterly coding window after approval is Thursday, October 1, 2026.
- Under the quarterly cycle, that October 1, 2026 filing targets an April 1, 2027 effective date for a permanent product-specific HCPCS code, assuming CMS keeps the application in-cycle.
- Between approval and permanent-code activation, providers will bill with interim codes such as C9399 or unclassified drug codes, plus full NDC, dosage, and narrative documentation.

OPPS transitional pass-through path

Milestone	Date	Operational meaning
ADRABETADEX PDUFA date from source pack	August 17, 2026	FDA approval is the gating event because CMS requires the NDA or BLA approval letter in the pass-through application.
Earliest post-approval pass-through filing deadline	September 1, 2026	A complete application received by the first business day in September can target a January 1, 2027 effective date.
Next pass-through filing deadline	December 1, 2026	A complete application received by the first business day in December shifts the earliest effective date to April 1, 2027.
Pass-through duration	2 to 3 years	CMS can make temporary additional OPPS payments for at least two years and no more than three years.

CMS accepts OPPS drug and biologic pass-through applications on an ongoing basis through MEARIS. The application requires the product description, dosage details, WAC and ASP information, hospital cost

information, date of first commercial sale, current HCPCS crosswalk or requested code descriptor, projected utilization by site of service, FDA approval letter, and the package insert.

Launch billing before the permanent code

- Hospital outpatient providers use interim codes such as C9399 for certain newly approved drugs or NOC codes when product-specific coding is not yet live.
- CMS contractor guidance for NOC drug billing requires drug name, NDC, dosage, and, when requested, narrative support that the billed use matches the labeled indication.
- A missing product-specific code does not prevent pass-through review, but it does increase denial risk and resubmission burden during the launch gap.
- The interim-coding gap therefore needs a payer-facing billing guide before approval, not after the first denials arrive.

Practical launch conclusion

ADRABETADEX can and should pursue both pathways in parallel. The fastest payment-support path is an immediate OPPOS pass-through application prepared for September 1, 2026 submission if approval lands on schedule. The permanent product-specific HCPCS path follows immediately after, with October 1, 2026 as the first post-approval quarterly coding target.

That sequencing yields a straightforward launch architecture: use interim billing codes at approval, pursue pass-through as the near-term outpatient payment bridge, and move to a permanent product-specific HCPCS code as soon as the quarterly cycle allows. The coding brief, the pass-through dossier, and the provider billing guide all need to exist before the August 2026 action date.

Decisions that need to be locked before approval

- Requested HCPCS descriptor and dose unit.
- Interim-code billing guide for hospitals, physician offices, and specialty centers.
- MEARIS pass-through dossier with WAC, projected utilization, and site-of-service projections ready for immediate filing.
- MAC and payer education package covering C9399 / NOC usage, NDC requirements, and clinical documentation.
- Internal rule for updating the provider net cost model when pass-through becomes effective or when the permanent HCPCS code goes live.

References

Source pack: Project Castar internal commercialization deck supplied in the source pack.

Source pack: Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for Adrabetadex supplied in the source pack.

Source pack: ADRABETADEX U.S. Reimbursement & Policy Deep Research Report supplied in the source pack.

UnitedHealthcare Miplyffa prior authorization criteria, effective April 1, 2025.

<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/h-p/PA-Notification-Miplyffa.pdf>

UnitedHealthcare Aqneursa prior authorization criteria, effective April 1, 2025.

<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/a-g/PA-Notification-Aqneursa.pdf>

HRSA 340B pricing formula page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/Pricing%20Formulas.htm>

HRSA new drug price estimation page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/New%20Drug%20Price%20Estimation.htm>

Medicaid.gov EPSDT page. <https://www.medicaid.gov/medicaid/benefits/early-and-periodic-screening-diagnostic-and-treatment>

Medicaid.gov PAD page.

<https://www.medicaid.gov/medicaid/prescription-drugs/state-prescription-drug-resources/physician-administered-drugs-pad>

Medicare and Medicaid quick guide, July 2024. <https://www.medicare.gov/publications/12207-a-quick-guide-medicare-medicaid.pdf>

CMS Medicare Part B ASP page. <https://www.cms.gov/medicare/payment/fee-for-service-providers/part-b-drugs/average-drug-sales-price>

CMS OPPS pass-through and new technology APC application page. <https://www.cms.gov/medicare/payment/prospective-payment-systems/hospital-outpatient/pass-through-payment-status-new-technology-ambulatory-payment-classification-apc>

CMS OPPS drug and biologic transitional pass-through instructions, updated March 16, 2026.

<https://www.cms.gov/files/document/determine-eligibility-drugs-biologicals-transitional-pass-through-payment-under-hospital-outpatient.pdf>

CMS HCPCS Level II coding decisions page.

<https://www.cms.gov/medicare/coding-billing/healthcare-common-procedure-system/current-prior-years-level-II-coding-decisions>

CMS 2025 first-quarter HCPCS drug and biologic coding determinations.

<https://www.cms.gov/files/document/2025-hcpcs-application-summary-quarter-1-2025-drugs-and-biologicals.pdf>